

A STORY OF AN ABANDONED OLD MAN

A story of an abandoned old man, one midnight encounter, and the responsibility we owe to the helpless.



“He called me Hanuman.

I could not be my idol Hanuman. I could only try to become worthy of his blessing by serving a helpless human being.”

A personal account of compassion, dignity, persistence—and a plea for a better system of immediate care for people found sick, abandoned and alone.

Prepared from the narrator’s account and photographs • August 2026

1. A Midnight Encounter

I was returning to my room after my usual night feeding round when I noticed an old man lying beside the road. It was around midnight. He appeared to be suffering from fever and severe pain. I went closer and offered him some chuda (flattened rice) and biscuits—the same food I normally carried in my bag for street dogs.

When I moved closer, I noticed a strong smell. Looking carefully, I discovered that his leg had a large, badly neglected wound that appeared to be heavily infected and infested with maggots. He was extremely weak and could not properly answer my questions.

I could not simply walk away. I decided to take him to the nearest Government Hospital. I lifted him in my arms and began carrying him. The journey from the HP petrol pump toward the Government Hospital was difficult; he was very heavy, and I had to stop several times to rest before continuing.



The condition of his leg was severe. The photograph is included as a record of the situation, not to sensationalize his suffering.

2. From the Hospital to NMCH—and Back

At the Government Hospital, I learned that concerned police personnel had already brought attention to him the previous day. Because his identity was unknown, he was being treated as a stranger. I requested that the hospital help us find a way to get him fully treated.

The casualty doctor advised me to return in the morning and speak to the doctors about referring him to NMCH, Patna, because the facilities available locally were not sufficient for the care he appeared to need. For that night, I requested that his wound at least be dressed. The staff did the dressing.

The next morning, an ambulance was arranged for the referral. I took leave from work and travelled with him as his attendant.

At NMCH, however, I felt helpless again. The severity of his condition and the strong odor made the situation difficult, and I was told, in effect, that I should take him back to my locality and care for him according to the instructions given. I had come hoping that a tertiary hospital would take over his care. Instead, I found myself facing the same question: Where can a person like this go when he has nobody?

It was a rainy day. I requested that we at least be taken back to Barh Government Hospital. After trying for about an hour, I was told that the drivers were occupied with other duties. So I brought him back to Barh by train.

3. A Safe Place Was Harder to Find Than Food

Back in Barh, I asked volunteers whether there was any safe place where I could keep him. A kind-hearted social worker helped me admit him to Barh Government Hospital. But he later left the hospital on his own, appearing confused and mentally disturbed.

I tried repeatedly to find a safe place for him, but he would leave whenever he could. So I began taking responsibility for him alongside my existing office work, rescued-dog care and street-animal feeding.

Every morning, I tried to give him around half an hour. Every night, I returned for another half hour. I looked for him, took food and water, checked his wound, and helped with dressing whenever necessary.

Mr. Aryan and his friends supported me financially during this period. I also learned about the “Roti Bank” group, which was providing food twice a day to abandoned and sick people living by the roadside. Their support and example reminded me that compassion becomes stronger when people work together.



A roadside shelter can protect a body for one night; dignity requires a system that can protect a person for as long as necessary.

4. The Daily Struggle

I kept wound-care materials with me—bandages, ointment, Betadine, cotton and other basic supplies. I also carried medicines that had been part of the care I was trying to provide. The most difficult problem, however, was not the dressing. It was finding him.

He often became soaked in the rain, which meant his clothes and bandages became wet. He also used khaini. His condition, confusion and frequent movement made it difficult to maintain continuity of care.

After about two weeks, he gradually began changing his location, sometimes moving around a kilometre away. Each day, finding him became harder. I sent emails to higher authorities after searching for official contact addresses and requested help. But nothing produced a lasting solution.

Then, after approximately two and a half weeks, he disappeared completely.

I searched with volunteers. We showed his photograph to people in different places. We tried to locate him, but ultimately we could not find him.



He was vulnerable not only because of his wound, but because he had no stable identity, shelter, attendant or continuous care.

5. “Hanuman” — and What He Taught Me

He used to call me “Hanuman.” Every time he said it, I felt both humbled and helpless.

I told myself: I cannot be my idol Hanuman. I am only an ordinary person. But I can pray for the blessing and strength to serve with an open hand—those who are needy, hopeless, helpless and abandoned, whether they are human beings, animals, birds or part of nature.

That is what this encounter taught me: compassion is not about being powerful enough to solve everything. Sometimes it is simply refusing to walk away when everyone else can.

He may have disappeared from my sight, but he should not disappear from our conscience.



This story is dedicated to every abandoned person who is still waiting beside a road, outside a hospital, under a staircase, or beneath a shop shutter—waiting for someone to notice.

6. What Should Happen When an Abandoned Person Is Found?

The experience raises a practical question: Can we create a simple response pathway so that a sick, abandoned and unidentified person does not have to depend on one passer-by?

A practical immediate-response model

1. Emergency medical assessment first.

If the person is seriously ill, unconscious, badly injured, severely infected, exposed to weather, or unable to care for himself, call emergency services and request medical transport.

2. Police / district control-room coordination.

Ask the police or district control room to record the location and the person's condition and to coordinate with the hospital and local administration.

3. Hospital social-work / administration referral.

Request that the hospital document the patient as an unidentified/abandoned person where appropriate, initiate treatment, and coordinate with the district administration rather than simply sending the person away.

4. Safe shelter + continuity of care.

For a person who is homeless, elderly, confused or recovering from serious illness, request linkage to an appropriate government/authorized shelter or social-welfare service.

5. Identity and family tracing.

Ask the police/local administration to begin lawful identity and family tracing, while protecting the person's dignity and privacy.

6. One accountable case coordinator.

The ideal system should assign one official or case worker who can coordinate hospital care, shelter, police tracing and follow-up.

Important: Do not delay emergency medical care while trying to complete paperwork. Also, antibiotics and prescription medicines should be given only under qualified medical supervision.

7. Official Contacts for Bihar / Patna District

The following contacts were checked against official Bihar Government / Patna District government webpages on 23 August 2026. For an urgent case, start with 112 or 102, then escalate through the district administration if the person is not receiving a safe, continuous plan of care.

Authority / service	Contact	Why contact them
National Emergency / Police	112	Immediate emergency response, police coordination and urgent protection.
Health / Ambulance	102 Health Dept: 1911 / 102	Urgent medical transport and health-system escalation.
Patna District Control Room	0612-2219810	District-level coordination for urgent local situations.
SDO, Barh	06132-242305 sdmbarh@gmail.com	Sub-divisional administrative escalation for a case in/around Barh.
District Magistrate, Patna	0612-2219545 dm-patna.bih@nic.in	Escalation when multiple departments must coordinate.
Civil Surgeon, Patna	9470003600	District health administration / medical coordination.
Social Welfare Directorate, Bihar	0612-2211718 dirdsw-bih@nic.in	Social-welfare support; the department's stated mandate includes old-age and deprived persons.
Social Security Directorate, Bihar	0612-2280373	Social-security / welfare escalation for vulnerable persons.
Chief Minister Public Grievance Office	0612-2205800	Escalation if a documented grievance is not being resolved.

Official sources: Patna District Administration (patna.nic.in), Bihar Social Welfare Department (state.bihar.gov.in/socialwelfare), and Patna District Helpline. Contacts can change; verify before relying on them for a non-emergency case.

8. A Small Request for a Big Change

I do not want this story to be remembered only as one man's struggle. I want it to become a question for our administration and our society:

When a helpless, abandoned and seriously ill person is found on a roadside, who is responsible for making sure he is safe tonight—and who follows his case tomorrow?

A simple protocol could save lives: emergency pickup → hospital assessment → police identity tracing → social-welfare/shelter placement → one assigned case coordinator → documented follow-up until the person is safely reunited with family or placed in appropriate long-term care.

People like this old man should not have to depend on whether a stranger happens to carry food, has a vehicle, has money, knows a doctor, or has enough time to search for them every day.

Compassion should be personal—but protection from abandonment should be institutional.



May every abandoned person be seen before it is too late. May every hospital see the human being before the difficulty of the case. And may every act of compassion become a reason to build a better system.

“I cannot be Hanuman. I can only try to live in a way that earns his blessing.”



Missing Complaint – Sudarshan Tiwari, Age 62+

1 message

Satyabrata Das <satya.ece05@gmail.com>
To: policehelpline-bih@gov.in

Tue, 26 Aug, 2025 at 02:47

Respected Sir/Madam,

I am writing to report the missing of Mr. Sudarshan Tiwari, a 62-year-old married but abandoned male residing at Kesath, Baijnathpur, District Bauxar, Bihar, 802125.

Mr. Tiwari has been suffering from a maggot-infected deep wound on his left leg upper foot. He was provided with food, water, and medical attention by local activists in Barh, under the prescription of NMCH, for the past week. He was last seen roaming near Medical Chowk, Barh, for about two weeks, receiving continuous help from activists.

However, for the last 30 hours, he could not be located by volunteers and all attempts to trace him have failed. He is reported missing since last Sunday afternoon near Medical Chowk Barh. His medical condition is serious and requires urgent attention.

We request that you kindly register this complaint and initiate all necessary action to find and safeguard Mr. Sudarshan Tiwari. His key identification details are as follows:

- Name: Sudarshan Tiwari
- Address: Kesath, Baijnathpur, District Bauxar, Bihar, 802125
- Age: 62+
- Social status: Married but abandoned
- Disease: Left leg upper foot maggot infected deep wound
- Last seen: Near Medical Chowk, Barh, Patna, Bihar
- Medical records: Under treatment as per NMCH prescription (can be provided)

Kindly treat this matter as urgent due to his vulnerable health and ensure his prompt search and protection.

Thank you for your kind attention and action.

Yours faithfully,

Thanks & Kind Regards

SATYABRATA DAS
satya.ece05@gmail.com
+91-9392940814/8095085855

4 attachments

-  **20250817_161722.jpg**
630 KB
-  **20250817_161712.jpg**
1.9 MB
-  **Screenshot_20250824_224817_WhatsAppBusiness.jpg**
487 KB
-  **IMG-20250817-WA0010.jpeg**
2 MB



Request for Immediate Assistance and Care for Abandoned Old Sick Homeless Elderly in Barh

2 messages

Satyabrata Das <satya.ece05@gmail.com>Fri, 22 Aug, 2025 at 04:05

To: sdmbarh@gmail.com

Respected Sir/Madam,

I am writing this application to bring to your urgent attention the distressing situation of several abandoned old human beings in Barh who are sick, homeless, hopeless, and helpless. These elderly individuals are in critical need of proper medical attention and basic care, yet remain neglected and vulnerable.

As concerned members of this community, we kindly urge your esteemed office to take immediate action to provide them with proper medical treatment, shelter, nutritious meals, and necessary social support. Many of these elderly are unable to take care of themselves and require urgent intervention to relieve their suffering.

We respectfully request that relevant government departments and healthcare staff be dispatched to assess their conditions and arrange for their admission to hospitals or care homes. Additionally, we urge coordination with welfare and social security schemes to ensure these vulnerable citizens receive long-term assistance.

Your timely intervention will greatly help restore dignity and hope to these neglected elderly and improve their quality of life.

Thank you for your attention to this critical matter. We hope for your compassionate and swift response.

Your's Sincerely
Satyabrata Das
9492940814/8095085855
satya.ece05@gmail.com

 My Report.PDF

36 KB

Satyabrata Das <satya.ece05@gmail.com>Fri, 22 Aug, 2025 at 04:14

To: sdmbarh@gmail.com

Thanks & Kind Regards
[Quoted text hidden]

8 attachments

-  20250817_161722.jpg
- 630 KB
-  20250817_161712.jpg
- 1.9 MB
-  20250820_235047.jpg
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-  20250821_233034.jpg
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3.7 MB

 **20250817_001751.jpg**
4.8 MB



Urgent Intervention and Medical Assistance Required for Abandoned Elderly Patient

1 message

Satyabrata Das <satya.ece05@gmail.com>
To: sdmbarh@gmail.com <sdmbarh@gmail.com>
Cc: dm-patna.bih@nic.in, A. Kumar <phcbarh2021@gmail.com>, sdhbarh@gmail.com <sdhbarh@gmail.com>

Tue, 19 Aug, 2025 at 11:36

Respected Sir/Madam,

I am writing to seek your immediate intervention regarding an elderly man, Mr. Sudarshan Tiwari (aged 62+, resident of Baidyanath Pur), who is abandoned and currently suffering from a severe, maggot-infested wound in his left foot.

Despite our efforts to get him treated at Barh Government Hospital and NMCH, he has not been admitted or given proper wound care. We have undertaken his wound cleaning and medication at our own initiative, but his status as a homeless individual makes it extremely difficult to ensure consistent treatment, shelter, food, and medical supervision.

- We earnestly request that you kindly:
- Arrange for his immediate admission to a government hospital for proper treatment,
 - Provide him with temporary shelter and basic needs like food and water,
 - Coordinate with health and welfare officials for long-term care.

We urge your kind consideration and swift action to save the life of this vulnerable citizen.

Thank you for your valuable support.

Thanks & Kind Regards
SATYABRATA DAS (Odisha)



SBD@SG A16 5G

Near V2 Mall, Barh, 803213
satya.ece05@gmail.com
+91-9392940814/8095085855

4 attachments

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-  **20250817_161712.jpg**
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-  **S Tiwari.PDF**
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